Nivolumab, ipilimumab, pemetrexed and carboplatin

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Cycle 1 Days 1 and 22 · chemotherapy-containing visits

Active medicines: nivolumab (Days 1 and 22) + ipilimumab (Day 1 only) + pemetrexed (Days 1 and 22) + carboplatin (Days 1 and 22)

Emetogenic risk: high

Medicine	Dose / route	When
Aprepitant	125 mg orally on treatment day; 80 mg orally on each of the following two days	Repeat the three-day course separately around Cycle 1 Day 1 and Day 22
Ondansetron	16 mg orally	Each chemotherapy treatment day Review QT risk, interactions and individual dose choice.
Antiemetic dexamethasone reference	12 mg orally on treatment day; 8 mg orally daily for the next three days	Repeat around both chemotherapy visits Reconcile with pemetrexed rash steroids and aprepitant interaction into ONE prescription; do not add schedules.
Metoclopramide	10 mg orally three times daily as prescribed	Subsequent days NCIS high-risk row does not define an end day or PRN qualifier; prescriber must specify duration, suitability and interaction risks.
Folic acid	350–1000 micrograms orally daily	At least five doses in seven days before first pemetrexed; throughout pemetrexed and for 21 days after its last dose
Vitamin B12 (hydroxocobalamin)	1000 micrograms intramuscular (IM)	Week before first pemetrexed; source repeat every three cycles only while pemetrexed continues This induction contains two pemetrexed administrations only. Do not automatically continue B12 during immune-only treatment.
Pemetrexed rash prophylaxis: dexamethasone equivalent	4 mg orally twice daily	Day before, treatment day and day after each pemetrexed administration Reference requirement: reconcile with antiemetic steroid into one clear prescription, not two added courses.

Additional phase-specific support

- Cycle 1: nivolumab, pemetrexed and carboplatin on Days 1 and 22; ipilimumab on Day 1 ONLY. Day 22 has no ipilimumab.
- Use dedicated carboplatin AUC ≥4 NCIS row; no routine olanzapine in that row.
- Folic acid starts before pemetrexed, continues through both chemotherapy visits and for 21 days after the last pemetrexed dose. B12 is given in the preceding week. The source repeats B12 every three cycles if pemetrexed continues; this planned course has only two pemetrexed administrations, so do not automatically continue B12 in immune-only cycles.
- Your team reconciles the steroid used to prevent pemetrexed rash with the anti-sickness steroid into one clear prescription. Do not add the two schedules together or use leftover steroids yourself.
- Nivolumab and ipilimumab can inflame healthy organs. Report new symptoms promptly, including after treatment stops; follow-up vigilance is required for at least five months after the last dose.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

National Cancer Control Programme (NCCP) 00713 v4 · pNCCP 00713 pp2–3,9–10; NCIS V8 Table 1 p4 · NCCP NCIS Medical Oncology antiemetics V8

Sources checked 2026-10-29 · SACTCheck v0.84.27 · Regimen-level drafts; prescriber and oncology-pharmacy verification required

Cycle 2 onwards Day 1 · nivolumab + ipilimumab every 42 days

Active medicines: nivolumab + ipilimumab

Emetogenic risk: low

Medicine	Dose / route	When
Ondansetron	8 mg orally	Day 1 before treatment, if following the NCIS low-risk default Individualise/local policy. No automatic antiemetic corticosteroid course.

Medicine	Dose / route	When
Metoclopramide	10 mg orally three times daily as needed	Subsequent days as prescribed Specify duration and check contraindications/interactions.

Additional phase-specific support

- Ipilimumab is low risk and nivolumab minimal; this combined visit is low risk.
- No further pemetrexed or platinum: stop/review induction antiemetics and rash steroid; complete the folate tail only.
- Continue immune symptom education, bloods each treatment and glucose/thyroid function tests (TFT) each 42-day cycle.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

National Cancer Control Programme (NCCP) 00713 v4 · pNCCP 00713 supportive care; NCIS V8 Table 1 p4 · NCIS low-risk row

Sources checked 2026-10-29 · SACTCheck v0.84.27 · Regimen-level drafts; prescriber and oncology-pharmacy verification required

Cycle 2 onwards Day 22 · nivolumab alone, between ipilimumab visits

Active medicines: nivolumab

Emetogenic risk: Minimal

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- No routine prophylactic antiemetic course for nivolumab-only minimal-risk treatment; prescribe symptom-directed rescue under local policy if needed.
- Ipilimumab is due at the next 42-day cycle Day 1, not every nivolumab visit.
- Immune toxicity can still occur between ipilimumab visits; the joint hold rule remains relevant.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

National Cancer Control Programme (NCCP) 00713 v4 · pNCCP 00713 schedule and emetogenic potential · Current NCCP regimen

Sources checked 2026-10-29 · SACTCheck v0.84.27 · Regimen-level drafts; prescriber and oncology-pharmacy verification required